CASOMB COMPLAINT FORM — CONTINUATION SHEET

Brendan Ngehsi Newanforbi v. HOPE Psychotherapy, Inc.

Date: February 21, 2026

I. NATURE OF COMPLAINT (Continued)

[Continued from Complaint Form, Section I. Items (a) through (c) appear on the form. Items (d) through (f) and the full narrative continue below.]

(d) Failure to maintain accurate records of client progress, including routine loss of homework assignments, in violation of Requirement 10;

(e) Maintenance of a treatment facility in unsanitary and unsafe conditions, in violation of Requirement 2 (basic operating policies and appropriate business conditions); and

(f) Financial conflict of interest undermining clinical objectivity, with revenue structures that incentivize indefinite client retention rather than treatment completion.

A. Blanket Retention Policy

HOPE maintains an official policy of refusing to certify "completion" for any parolee prior to discharge from parole, regardless of clinical progress, compliance, risk reduction, or time in treatment. This policy was documented under oath by HOPE's own Executive Director of Clinical Training in the case of *In re Frederick Dwyer*, No. SC-080355A (San Mateo Superior Court, March 20, 2023), where the court found this practice "vitiates the Legislature's directive" under Penal Code § 3008(d)(2). Despite this judicial condemnation, Ms. Palacios has continued applying the identical policy to my treatment through November 2025 and beyond.

Effective April 17, 2025, Title 15 CCR § 3574(f)(1) explicitly provides that "Treatment completion and parole discharge are legally separate decisions." A treatment provider that refuses to certify completion until discharge from parole collapses these separate decisions into one, in direct violation of both this regulation and CASOMB's own Treatment Completion Guidelines (2018), which require individualized assessment of risk level, criminogenic needs, and individual factors.

Ms. Palacios represented to CDCR—through treatment status reports, CTM inputs, and the November 2025 Discharge Review Report—that I have "not completed" sex offender treatment. This characterization was not the product of an individualized clinical assessment. It was the automatic output of HOPE's blanket retention policy, which makes completion impossible for every participant prior to parole discharge without exception.

B. Retaliatory and Defamatory Statements at CTM Meeting

In or about August 2025, during a Case Conference/Containment Team Meeting ("CTM") held at a CDCR parole office in Stockton, California, Ms. Palacios made false, defamatory, and retaliatory statements about me in the presence of multiple parole agents. The following individuals were present: Agent Chow (Parole Agent, CDCR); Agent G. Noguchi #7284 (Supervising Parole Agent, CDCR); Agent A. Derrick #6910 (my assigned Parole Agent, CDCR); the then-current supervising parole agent (now retired); and Ms. Palacios.

After initially acknowledging my respectful conduct during the meeting, Ms. Palacios stated, in substance, that I was "disruptive during HOPE group sessions," a "ringleader," that my behavior

was “abhorrent,” and that I was “often disrespectful to [her] staff.” She characterized my respectful demeanor at the CTM as an “acting job.”

These statements were false. My conduct in group sessions consisted of raising legitimate concerns about program conditions, including: my prolonged retention in the program despite completion of all assigned coursework; the repeated loss of my completed homework by program staff; and the disruption of my progress caused by repeated clinician turnover and resulting group resets. Other participants who independently shared similar concerns were falsely attributed to my “influence.” Ms. Palacios’s own words confirm the retaliatory nature: “because when you start complaining then the other guys chime in”—i.e., others voluntarily agreed with my protected expression.

Following this incident, I curtailed my speech during HOPE group sessions, abandoned written complaints, and experienced significant anxiety and emotional distress. Ms. Palacios subsequently praised me for the change—confirming that her prior statements were designed to suppress advocacy and punish protected expression.

C. Arbitrary Treatment Structure With No Published Criteria

Under Ms. Palacios’s direction, HOPE’s treatment structure requires mandatory phase repetitions with no basis in statute, regulation, or CASOMB requirements. Participants must complete the “Core” phase twice, the “Maintenance” phase twice, and then a “titration” phase before being considered for program completion. This framework appears nowhere in Penal Code § 3008(d) or Title 15 CCR § 3574, and results in participants remaining in treatment for 5–7+ years with no meaningful prospect of completion.

Ms. Palacios has never provided me with: (a) written criteria for program completion; (b) a written explanation of what conduct or progress is required to advance through phases; (c) an appeal process for titration or phase-advancement decisions; or (d) any mechanism to challenge lost homework or forced phase restarts.

D. Failure to Maintain Accurate Records

Ms. Palacios has admitted that homework assignments are “routinely lost” within the HOPE Program. Despite this admission, she has failed to implement any safeguards against record loss. Participants—including me—are forced to restart treatment phases due to staff turnover or job-related group changes, forfeiting progress previously earned. This constitutes a failure to maintain accurate and complete documentation of client progress.

E. Unsanitary and Unsafe Facility Conditions

The HOPE Program facility at 5403 N. Pershing Avenue, Suite 200, Stockton, CA 95207 is maintained in conditions that are inconsistent with a professional treatment environment. Photographic evidence (attached as Exhibits) documents the following conditions at the facility:

> **Restroom Conditions:** Severely unsanitary restrooms including clogged and overflowing toilets filled with waste and debris; dirty and stained tile floors with visible grime; trash and food packaging dumped in sinks; unclean countertops with spills and biological matter. (See Exhibits: IMG_4599 through IMG_4612.)
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> **Stairwell and Building Access:** Dark, poorly lit, or completely unlit interior stairwells that present a safety hazard, particularly for clients attending evening treatment sessions.

The exterior entrance areas are neglected and contain litter. (See Exhibits: IMG_4597, IMG_4598, IMG_4613, IMG_4614.)

These conditions exist at a facility that bills $100.00 per 90-minute group psychotherapy session, $200.00 per 60-minute individual session, $500.00 per polygraph examination, and $1,800.00 per psychosexual evaluation. The contrast between the fees charged and the conditions maintained raises serious concerns about the agency's commitment to providing professional treatment services.

F. Financial Conflict of Interest

HOPE operates under a fee-for-service billing model. Per the Alameda County Probation Department Procurement Contract No. 26374 (First Amendment approved January 9, 2024), HOPE bills: $100.00 per group psychotherapy session (90 minutes); $100.00 per Collaborative Model Team meeting (per client staffed, 90 minutes); $200.00 per individual psychotherapy session (60 minutes); $500.00 per Polygraph Examination; $1,800.00 per psychosexual evaluation; and $125.00 per quarterly treatment plan.

A single parolee attending two group sessions per week generates approximately $10,600 annually in baseline group and CTM fees alone. Over a 10-year parole term, this represents approximately $106,000 in baseline revenue from continued enrollment. HOPE's contracts provide no documented compensation, bonus, or incentive for successful treatment completion. Revenue terminates upon discharge. Ms. Palacios, as Clinical Director, controls the "completion" determination that CDCR uses as a gatekeeper for discharge eligibility, while possessing a direct, ongoing financial interest in ensuring that determination is never favorable.

II. STANDARD(S) VIOLATED (Continued)

[Continued from Complaint Form, Section II. The form contains the beginning of Requirement 2, Item 4. The full text of all violated requirements continues below.]

REQUIREMENT 2: Basic Operating Policies and Practices

Item 4 — Compliance with Ethical Standards (ATSA Code of Ethics): Ms. Palacios made false, defamatory, and retaliatory statements about me at a CTM meeting in the presence of parole officials, in direct response to my raising legitimate complaints about program conditions. She later confirmed the retaliatory nature by praising me when I ceased exercising those rights. Retaliating against a client for protected expression violates the ethical standards required of treatment providers, including the ATSA Code of Ethics.

Item 5 — Treatment Provider Boundaries: Using the therapist-client relationship to punish protected speech and suppress advocacy constitutes an abuse of professional authority and a violation of treatment provider boundaries with offender clients.

Item 10 — Monitoring Compliance with CASOMB and SARATSO Requirements: HOPE has failed to monitor its own compliance with CASOMB requirements, including the Treatment Completion Guidelines, the Maintenance and Aftercare guidelines, and the requirement for individualized treatment planning.

General — Appropriate Business Conditions: The CASOMB Certification Requirements state that agencies must have "appropriate business licenses for their locations." The HOPE facility is maintained in severely unsanitary and unsafe conditions (unsanitary restrooms, dark unlit stairwells, neglected premises) that are inconsistent with a professional treatment environment and raise questions about compliance with basic health and safety standards. Photographic evidence is attached.

REQUIREMENT 5: Informed Consent, Waiver of Confidentiality, and Treatment Contract

Section A, Item 12 — Client Fees: HOPE operates under a fee-for-service billing model that creates a structural financial conflict of interest. Revenue terminates upon discharge, providing a direct financial incentive to retain clients indefinitely rather than certify treatment completion. This conflict was not disclosed to me.

Section D — Treatment Contract (Items 1 and 5): The treatment contract must "describe the type, frequency, and requirements of the treatment, and outline how the duration of treatment will be determined" (item 1) and "describe the provider agency's grievance process to address and resolve client complaints" (item 5). HOPE failed to provide written criteria for treatment duration, phase progression, or completion. HOPE also failed to provide a meaningful grievance process—raising complaints about program conditions resulted in retaliation rather than resolution.

REQUIREMENT 7: Written Treatment Plan

CASOMB requires individualized written treatment plans. HOPE applied a blanket retention policy—making completion impossible for all participants prior to parole discharge—rather than maintaining individualized treatment plans with achievable goals and measurable completion criteria. My treatment status characterization was the automatic output of this blanket policy, not the product of individualized clinical assessment. This violates both the

requirement for individualized written treatment plans and the CASOMB Treatment Completion Guidelines (2018), which require that "the determination of treatment completion" consider risk level (Guiding Principle 11), individual factors (Principle 12), predicted trajectory (Principle 13), and containment team judgment (Principle 14).

REQUIREMENT 9: Treatment (Including Treatment Completion)

Treatment Completion Guidelines (2018): HOPE's blanket retention policy violates the Treatment Completion Guidelines, which require individualized assessment based on risk level, criminogenic needs, and the RNR (Risk, Need, Responsivity) principles. The guidelines explicitly state that treatment completion is a "protective factor" associated with reduced risk (Principle 16), and that the determination should be a "YES-or-NO conclusion about whether treatment has been completed" (Principle 20).

Maintenance and Aftercare Guidelines (September 2024): The Maintenance and Aftercare guidelines require that "discharge planning starts at intake" and that "the final determination regarding whether an individual has 'completed' treatment should never come as a surprise to the client." HOPE's blanket retention policy, combined with its unpublished criteria, ensures that completion is both a surprise and an impossibility.

REQUIREMENT 10: Other Documentation

Ms. Palacios has admitted that homework assignments are "routinely lost" within the HOPE Program. Despite this admission, no safeguards have been implemented. Participants are forced to restart treatment phases due to staff turnover or lost work, forfeiting progress previously earned. This constitutes a failure to maintain accurate and complete documentation of client progress as required by CASOMB certification standards.

REQUIREMENT 11: Complaints Against Treatment Providers and/or Provider Agencies

Requirement 11 mandates that the provider agency maintain an internal grievance procedure and notify offender clients of the CASOMB complaint process. Ms. Palacios retaliated against me for raising legitimate complaints about program conditions during group sessions, thereby chilling the exercise of the grievance and complaint processes that CASOMB requires agencies to maintain. Per the CASOMB Complaint Procedure (2025), item 19: "Respondent providers or provider agencies may not discriminate or retaliate against complainants who filed a complaint in good faith."

III. STEPS TAKEN TO RESOLVE THE COMPLAINT (Continued)

[Continued from Complaint Form, Section III. Items 1-2 begin on the form. The full list continues below.]

1. I raised my concerns about program conditions directly during HOPE group sessions in 2024 and 2025, including my prolonged retention, the loss of homework assignments by program staff, and the disruption caused by clinician turnover and group resets. Rather than address these concerns, Ms. Palacios retaliated by making defamatory statements about me at the August 2025 CTM meeting.

2. Following the retaliation, I curtailed my speech during group sessions and abandoned further written complaints to HOPE, as the retaliation made clear that internal complaint mechanisms would result in further adverse consequences.

3. I filed a complaint with the California Board of Behavioral Sciences (BBS) against Ms. Palacios (LMFT License No. 44555) regarding her professional misconduct.

4. I filed a federal civil rights action: *Newanforbi v. Derrick, et al.*, Case No. 2:26-cv-00004-DJC-JDP (E.D. Cal.), which includes claims arising from the same course of conduct.

5. I am now filing this CASOMB complaint as the appropriate forum for addressing violations of the CASOMB Provider Agency Certification Requirements.

IV. DESIRED OUTCOME (Continued)

[Continued from Complaint Form, Section IV. Items 1-2 begin on the form. The full list continues below.]

1. Investigate HOPE's blanket retention policy and determine whether it violates the CASOMB Treatment Completion Guidelines, the Maintenance and Aftercare guidelines, and the Provider Agency Certification Requirements for individualized treatment planning.

2. Conduct an on-site compliance review of the HOPE Program Stockton facility, including an inspection of facility conditions (restrooms, stairwells, common areas) and a review of client files for compliance with individualized treatment planning and documentation requirements.

3. Require HOPE to provide all current participants with written, achievable criteria for phase progression, treatment completion, and any appeal or challenge mechanisms, in compliance with the Treatment Contract requirements (Requirement 5, Section D) and Written Treatment Plan requirements (Requirement 7).

4. Require HOPE to implement safeguards against the routine loss of client homework and progress documentation, and to ensure that participants are not forced to restart treatment phases due to the agency's administrative failures.

5. Require HOPE to bring the Stockton facility into compliance with professional standards for a treatment environment, including sanitary restrooms and adequately lit stairwells and common areas.

6. Take appropriate disciplinary action against HOPE and/or Ms. Palacios for the violations identified herein, which may include a letter of admonition, probation, suspension, or decertification as warranted by the findings.

7. Refer any findings of criminal and/or unethical behavior to the appropriate licensing board (California Board of Behavioral Sciences) and/or law enforcement agency.

V. SUPPORTING DOCUMENTATION AND EXHIBITS

The following supporting documentation and evidence is attached to or accompanies this complaint:

Exhibit A: Photographic evidence of unsanitary restroom conditions at the HOPE Stockton facility (IMG_4599 through IMG_4612)

Exhibit B: Photographic evidence of dark, unlit stairwells and neglected building exterior (IMG_4597, IMG_4598, IMG_4613, IMG_4614)

Exhibit C: HOPE Program Fee Model documentation

Exhibit D: Discharge Review Report (November 2025)

Exhibit E: BBS Complaint against Katherine A. Palacios, LMFT License No. 44555

Exhibit F: *First Amended Complaint in Newanforbi v. Derrick, et al.*, Case No. 2:26-cv-00004-DJC-JDP (E.D. Cal.)

Thank you for addressing these concerns to the Board. I am available to provide any additional information or documentation required to investigate this complaint.

Respectfully submitted,

/s/ Brendan Ngehsi Newanforbi
February 21, 2026